Question:

A 34-year-old woman presents to her OB/GYN with complaints of missing her last 3 periods as well as intermittent spontaneous milky-white nipple discharge bilaterally for the past 3 months. Vital signs are stable and within normal limits. Neurologic examination is without abnormality, including normal visual fields. Serology and MRI of the brain are ordered, with results pending. Which of the following sets of laboratory results would be expected in this patient?

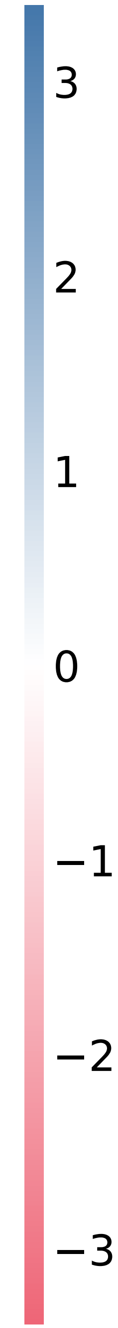
Answer Choices:

A. Decreased prolactin, decreased FSH, decreased LH

B. Decreased prolactin, increased FSH, increased LH

C. Increased prolactin, decreased FSH, increased LH

D. Increased prolactin, decreased FSH, decreased LH



Reasoning + Answer:

<think> Reasoning process here: The patient's symptoms of amenorrhea and galactorrhea (milky-white nipple discharge) are highly suggestive of hyperprolactinemia, which is often associated with a prolactinoma, a type of pituitary tumor. Neurological examination and imaging studies are normal, which may rule out more severe central nervous system causes of hyperprolactinemia such as stalk-mediating lesions or lesions that affect the brain's hypothalamus. The absence of neurologic deficits suggests that there is not a significant central nervous system problem. Serum prolactin levels are likely to be elevated as the primary cause of this patient's symptoms. FSH and LH levels could either be low or normal, as prolactin inhibits the secretion of these gonadotropins.</think><answer>C. Increased prolactin, decreased FSH, increased LH